

Bay Area Orthopedic Specialists, Inc.

Reza Tehrani, MD — Spine & Orthopedic Surgery
3010 Webster Street, Oakland, CA 94609

CONSULTATION NOTE

Patient Name: David Park
Date of Birth: September 24, 1986
Account Number: BAOS-44712
Date of Visit: June 18, 2024
Referring MD: Marcus Hayes, MD (East Bay Family Medicine)

REASON FOR CONSULTATION

Right-sided low back pain with intermittent radiculopathy, persistent ~5 weeks despite conservative management.

HISTORY

Mr. Park is a 38-year-old right-handed software engineer who presents with persistent low back pain following an acute mechanical injury sustained while moving boxes on May 12, 2024. He was initially evaluated at urgent care and treated with NSAIDs and cyclobenzaprine. Pain partially improved but did not resolve, and over the past 2-3 weeks he has developed intermittent radiating sensations down the posterior right thigh, with brief episodes of an "electric" or "buzzing" quality. No frank weakness reported. No saddle anesthesia. No bowel or bladder dysfunction.

His occupation involves prolonged sitting (8-10 hours daily), and he reports symptoms are significantly worse with sitting and improved with walking and standing. Cough and sneeze do not clearly worsen the pain.

He has tried: ibuprofen 600 mg up to QID, naproxen 500 mg BID for the past 2 weeks, heating pad, ergonomic adjustment to standing desk, and avoidance of heavy lifting. He has not tried physical therapy.

PMH/PSH: unremarkable. No prior spine surgery, no spinal injections.

PHYSICAL EXAMINATION

Gait is non-antalgic. He sits with mild discomfort and shifts position frequently. Trunk ROM demonstrates approximately 60 degrees of forward flexion (limited by reproduction of right-sided back pain), full extension, and side bending. Spinal palpation reveals tenderness at the right L4-L5 and L5-S1 paraspinal regions without midline tenderness.

Neurologic examination of the lower extremities:

Motor: 5/5 throughout, including hip flexors, quadriceps, tibialis anterior, EHL, and gastrocnemius bilaterally.
Sensory: Intact to light touch in all dermatomes bilaterally. Patient reports a subjective "vague" sensory difference along the lateral aspect of the right calf compared to the left, but two-point discrimination is intact.
Reflexes: Patellar 2+ bilaterally; Achilles 2+ on the left, 1+ on the right (mild asymmetry).
Special tests: Right straight-leg raise positive at 50 degrees with reproduction of right-sided radicular symptoms into the posterior thigh. Left SLR negative. Crossed SLR negative.

ASSESSMENT

Right-sided lumbar radiculopathy, most consistent with L5 or S1 nerve root involvement, in the setting of a likely disc-related process. Symptoms have persisted beyond 6 weeks and are now limiting his function. Imaging is indicated to define the anatomy and guide treatment.

PLAN

1. MRI lumbar spine without contrast — will be ordered today, scheduled at NorCal Imaging Center.
2. Initiate physical therapy twice weekly for 6 weeks; will provide referral to Active Recovery PT.
3. Continue current NSAID regimen.
4. Discussed treatment trajectory: most disc-related radiculopathies respond to conservative management within 6-12 weeks. If symptoms persist or progress despite PT and imaging-guided management, options include epidural steroid injection or, in cases of persistent or progressive neurologic deficit, surgical decompression.
5. Return for follow-up after MRI is completed and PT has begun, approximately 4 weeks.

Total time spent with patient: 35 minutes, of which more than 50% was counseling and care coordination.

Sincerely,

Reza Tehrani, MD
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